

2026 Summary Plan Description - demonstration edition

P1 | 1. Plan identity

SYNTHETIC / DEMONSTRATION DATA. Fictional abbreviated SPD, not a complete legally compliant benefit booklet. Plan sponsor: Lumen Meadow Design LLC. Plan ID: LM-2026-EPO. Administrator: Lumen Meadow Benefits Committee. Claims administrator: Harborlight Benefit Services (fictional). This is a private, single-employer self-funded ERISA group health plan, not an insurance policy issued by Harborlight. The plan is non-grandfathered. Plan year January 1 through December 31, 2026. California member residence does not change this funding designation.

P2 | 2. Member and eligibility

Member: Avery Rowan. Member ID: SYN-M1042. Individual coverage effective January 1, 2026; active through September 24, 2026 in this demonstration. Member services: benefits@harborlight.example (non-operational). Group: SYN-LM26. No other health coverage is recorded, but members must report any other coverage.

P3 | 3. Enrollment administration

Address and dependent updates may be submitted through the demonstration member portal. Keep a copy of notices. This excerpt includes relevant benefit terms and selected unrelated benefits. A real reviewer should obtain all controlling plan documents and amendments.

Benefits schedule

P4 | 4. Individual medical benefits

Annual in-network deductible: \$1,000. Annual in-network out-of-pocket maximum: \$5,000. Covered outpatient surgery facility services and associated anesthesiology: 20% member coinsurance after the deductible; no separate copayment. Applicable statutory cost-sharing limits and exceptions control. The deductible and out-of-pocket ledger on the EOB shows processing status, not proof of payment.

P5 | 5. Other services

Primary-care office visit: \$25 copayment. Specialist office visit: \$40 copayment. Covered preventive services use the preventive-benefit schedule. Outpatient physical therapy: 20% after deductible, ordinarily 20 visits per plan year subject to the plan review process. Retail pharmacy: generic tier \$10; preferred brand tier \$35. Dental cleaning and routine adult vision hardware are outside this medical demonstration benefit.

P6 | 6. Coordination and reimbursement

When other coverage exists, the administrator will request details before determining payer order under the controlling coordination provisions. Itemized receipts may be requested for member-submitted reimbursement. This abbreviated booklet does not establish a universal coordination rule.

Network and facility exceptions

P7 | 7. General network restriction

Non-emergency services by out-of-network providers are excluded unless an exception in Section 8 or another controlling requirement applies. Network status must be determined for the specific provider or facility and the service date. A participating facility does not necessarily mean every clinician participates.

P8 | 8. Protected facility services

Covered non-emergency anesthesiology furnished by a nonparticipating provider during a visit to a participating hospital, hospital outpatient department or ambulatory surgical center is handled using applicable federal surprise-billing protections. For those services, the plan applies in-network cost sharing based on the legally applicable recognized amount and credits applicable in-network accumulators. Provider reimbursement is determined separately. An ordinary office visit is not automatically a visit to a qualifying facility.

P9 | 9. Provider charges

Protected cost sharing can still be owed. A bill alone does not determine the legally collectible balance. If an item is disputed, ask for the submitted claim, service location and an explanation of benefit calculations. The patient should not determine or change medical billing codes without the provider reviewing the actual services.

Authorization and coverage administration

P10 | 10. Authorization scope

Scheduled outpatient knee arthroscopy requires advance review. Associated anesthesia for an approved covered surgery does not require a separate authorization under this fictional plan. Approval applies to the specified service, facility and date range. It does not establish the actual place of service, continued enrollment, claim coding or final payment amount.

P11 | 11. Exclusions and review

Purely cosmetic services and noncovered convenience items are excluded under this demonstration plan. Medical necessity, benefit exclusions and a missing authorization are different review issues. An adverse notice should identify the basis for its decision. Do not infer a clinical denial from an administrative or network denial.

P12 | 12. Claim submission

For this demonstration plan, initial claims ordinarily must be submitted within 365 days of service, subject to applicable exceptions. This is a fictional plan term, not a federal filing deadline. Members may obtain help from the claims administrator if a provider cannot submit. Original receipts should be retained.

Member claims and appeal rights

P13 | 13. Internal appeal

This plan has one mandatory internal appeal level. Submit a written appeal within 180 calendar days after receiving the adverse determination. For the demonstration deadline calculation, exclude the receipt date and add 180 days; submit early and retain proof. Identify the member, claim, disputed decision, requested remedy and supporting records. Use the demo appeal intake at appeals@harborlight.example; this address is fictional and must never receive real records. A correction request does not extend the appeal deadline under these plan terms.

P14 | 14. Review and records

The plan will provide relevant claim records free on request. A new reviewer will consider the appeal. The plan provides a post-service appeal decision within 60 days after receipt of the appeal. Ask for copies of the submitted claim data, applicable rules and the basis for the location classification.

P15 | 15. Further review and help

Eligible unresolved claims may use the applicable federal external-review process administered through a contracted independent review organization. The final internal adverse notice will provide instructions and the deadline. This plan is not assigned to the HHS-administered process in this demo. Contact the plan administrator or U.S. DOL EBSA for help with plan claims rights; applicable provider surprise-billing concerns can also be raised with the CMS No Surprises Help Desk. This excerpt is demonstration material, not an actual notice of rights.